Subject: FW: PLOS ONE Decision: Revision required [PONE-D-26-27576] - [EMID:57cc48cd17d4f67a]
Date: Thursday, July 30, 2026 at 8:06:13 AM Eastern Daylight Time
From: Gregory Jaffe
To: Renee Walker
CC: suzan walters

Hey Renee,

Great news! Revise and resubmit!

I was wondering whether you could comment on the "black box" comment from the reviewers?

Thanks much friend,
Greg

From: em.pone.0.9d1916.76c24148@editorialmanager.com <em.pone.0.9d1916.76c24148@editorialmanager.com> on behalf of PLOS ONE <em@editorialmanager.com>
Sent: Thursday, 30 July 2026 02:24:46
To: Suzan Walters <suzanmwalters@gmail.com>
Subject: PLOS ONE Decision: Revision required [PONE-D-26-27576] - [EMID:57cc48cd17d4f67a]

PONE-D-26-27576
Reducing Stigma Toward People Who Use Substances Through a Pop-Up Shop Intervention: A Mixed-Methods Pre–Post Study
PLOS One

Dear Dr. Walters,

Thank you for submitting your manuscript to PLOS ONE. After careful consideration, we feel that it has merit but does not fully meet PLOS ONE's publication criteria as it currently stands. Therefore, we invite you to submit a revised version of the manuscript that addresses the points raised during the review process.

While the reviewers recognized the innovative nature of your community-based intervention, significant concerns were raised regarding the study's methodology, statistical analysis, and the presentation of the qualitative data.

In addition to addressing all comments from both reviewers, please ensure the following critical editorial points are thoroughly resolved in your revision:

1. Attrition Analysis: The study reports that 70 participants completed the pre-survey, but only 39 completed the matched pre-post surveys. You must conduct and report an attrition analysis

comparing the baseline attitudes of the 39 participants who completed both surveys against the 31 who dropped out. This is essential to evaluate potential non-response bias.

2. Appropriateness of Statistical Tests: Paired t-tests were used to assess changes in stigma-related beliefs measured on a 5-point Likert scale. Since Likert data is ordinal, non-parametric tests (such as the Wilcoxon signed-rank test) are methodologically more appropriate. Please re-analyze these outcomes. These non-parametric tests can be rapidly executed using standard libraries in R or Python. If you choose to retain the t-tests, you must robustly justify the assumption of normality for this specific dataset.

3. Correction for Multiple Comparisons: Your analysis involves multiple independent statistical tests on the same cohort (e.g., eight variables compared in Table 2). To mitigate the risk of Type I errors (false positives), please apply an appropriate correction for multiple comparisons (e.g., Bonferroni or Holm-Bonferroni method) and update your significance thresholds and p-values accordingly. Direct implementations of these corrections are readily available in standard statistical environments like R.

4. Self-Selection Bias and Demographics: As noted by the reviewers, the lack of demographic data prevents the control of confounding variables. Furthermore, because participants voluntarily approached the harm reduction booth, self-selection bias heavily impacts the external validity of the findings. These limitations must be explicitly expanded upon in the Discussion section.

5. Intervention Description ("Black Box"): Please provide a much more detailed and transparent description of the intervention components and implementation. Clarify the nature of the interactions with study staff and specify how the items (e.g., apparel, kits) were distributed or sold.

6. Data Availability: Your current Data Availability statement ("corresponding author") does not comply with PLOS ONE's open data policy. Please ensure your de-identified dataset is deposited in a public repository or provided as Supporting Information.

We look forward to receiving your revised manuscript and a detailed point-by-point response to all reviewer and editorial comments.

Please submit your revised manuscript by Sep 28 2026 11:59PM. If you will need more time than this to complete your revisions, please reply to this message or contact the journal office at plosone@plos.org. When you're ready to submit your revision, log on to https://www.editorialmanager.com/pone/ and select the 'Submissions Needing Revision' folder to locate your manuscript file.

Please include the following items when submitting your revised manuscript:

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- A marked-up copy of your manuscript that highlights changes made to the original version. You should upload this as a separate file labeled 'Revised Manuscript with Track Changes'.
- An unmarked version of your revised paper without tracked changes. You should

upload this as a separate file labeled 'Manuscript'.

If you would like to make changes to your financial disclosure, please include your updated statement in your cover letter. Guidelines for resubmitting your figure files are available below the reviewer comments at the end of this letter.

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We look forward to receiving your revised manuscript.

Kind regards,

Iara do Nascimento Teixeira, Ph.D
Academic Editor
PLOS One

Journal Requirements:

When submitting your revision, we need you to address these additional requirements.

1. Please ensure that your manuscript meets PLOS ONE's style requirements, including those for file naming. The PLOS ONE style templates can be found at

https://journals.plos.org/plosone/s/file?id=wjVg/PLOSOne_formatting_sample_main_body.pdf
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2. Please provide additional details regarding participant consent. In the ethics statement in the Methods and online submission information, please ensure that you have specified (1) whether consent was informed and (2) what type you obtained (for instance, written or verbal, and if verbal, how it was documented and witnessed). If your study included minors, state whether you obtained consent from parents or guardians. If the need for consent was waived by the ethics

committee, please include this information.

3. Thank you for stating the following in your Competing Interests section:

“None”.

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6. Please include your full ethics statement in the ‘Methods’ section of your manuscript file. In your statement, please include the full name of the IRB or ethics committee who approved or waived your study, as well as whether or not you obtained informed written or verbal consent. If consent was waived for your study, please include this information in your statement as well.

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9. We note that Figures 1, 2 and 3 includes an image of a participant in the study.

As per the PLOS ONE policy (http://journals.plos.org/plosone/s/submission-guidelines#loc-human-subjects-research) on papers that include identifying, or potentially identifying, information, the individual(s) or parent(s)/guardian(s) must be informed of the terms of the PLOS open-access (CC-BY) license and provide specific permission for publication of these details under the terms of this license. Please download the Consent Form for Publication in a PLOS Journal (http://journals.plos.org/plosone/s/file?id=8ce6/plos-consent-form-english.pdf). The signed consent form should not be submitted with the manuscript, but should be securely filed in the individual's case notes. Please amend the methods section and ethics statement of the manuscript to explicitly state that the patient/participant has provided consent for publication: "The individual in this manuscript has given written informed consent (as outlined in PLOS consent form) to publish these case details".

If you are unable to obtain consent from the subject of the photograph, you will need to remove the figure and any other textual identifying information or case descriptions for this individual.

10. If the reviewer comments include a recommendation to cite specific previously published works, please review and evaluate these publications to determine whether they are relevant and

should be cited. There is no requirement to cite these works unless the editor has indicated otherwise.

[Note: HTML markup is below. Please do not edit.]

Reviewers' comments:

Reviewer's Responses to Questions

Comments to the Author

1. Is the manuscript technically sound, and do the data support the conclusions?

The manuscript must describe a technically sound piece of scientific research with data that supports the conclusions. Experiments must have been conducted rigorously, with appropriate controls, replication, and sample sizes. The conclusions must be drawn appropriately based on the data presented.

Reviewer #1: Yes

Reviewer #2: Partly

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2. Has the statistical analysis been performed appropriately and rigorously?

Reviewer #1: Yes

Reviewer #2: Yes

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3. Have the authors made all data underlying the findings in their manuscript fully available?

The PLOS Data policy requires authors to make all data underlying the findings described in their manuscript fully available without restriction, with rare exception (please refer to the Data Availability Statement in the manuscript PDF file). The data should be provided as part of the manuscript or its supporting information, or deposited to a public repository. For example, in addition to summary statistics, the data points behind means, medians and variance measures should be available. If there are restrictions on publicly sharing data—e.g. participant privacy or use of data from a third party—those must be specified.

Reviewer #1: Yes

Reviewer #2: No

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4. Is the manuscript presented in an intelligible fashion and written in standard English?

PLOS ONE does not copyedit accepted manuscripts, so the language in submitted articles must be clear, correct, and unambiguous. Any typographical or grammatical errors should be corrected

at revision, so please note any specific errors here.

Reviewer #1: Yes

Reviewer #2: Yes

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5. Review Comments to the Author

Please use the space provided to explain your answers to the questions above. You may also include additional comments for the author, including concerns about dual publication, research ethics, or publication ethics. (Please upload your review as an attachment if it exceeds 20,000 characters)

Reviewer #1: Comments for the Authors:

This paper addresses a difficult topic to study and to manage in clinic, i.e. stigma towards people who use drugs (PWUD). Using a single intervention, a 4 hour pop-up fashion clinic on an urban college campus, the authors assess pre- and post-intervention knowledge and attitudes towards PWID among 39 participants who completed brief pre and post event surveys; 70 people completed pre-intervention experiences. The authors used a mixed-methods approach using visual and experiential engagement via personal stories and anecdotes from people with experience with addiction and recovery. The results were mixed, with some improvements in feelings towards PWUD and interaction with them, and a reduction in the association between drug use and unpredictability. Two thirds of respondents reported treating individuals equally regardless of whether or not they were PWUDs. No changes were seen in the frequency of reports that PWUDs make poor decisions and are dangerous. Among participants, approval of the intervention was high. There are several questions that require attention, as well as further explanation of choices in the methodology.

The authors should clarify a few points. The respondents should be better clarified by demographics, particularly age, sex, race/ethnicity, and educational level. Even better would be clarification on past drug use or familiarity with PWUDs, if available.

The role of fashion in the pop-up event is not described, and might shed light on the audience that was attracted to the event. Was an explicit connection between fashion and drug use made, or implied, e.g. with the choice of vendors and clothing, or potential sponsors? Why fashion and not sports, or mental health?

More information is needed about the decision making on the choice of venue, time of day, etc. Was this on a weekend or a work day? Why was this college campus chosen?

The target audience composition needs clarification, i.e. are Gen Z youth the most important group to have buy in for stigma reduction? Their attitudes towards substance use and harm reduction are already fairly positive, and this might have contributed to the lack of statistically significant changes in attitudes post-intervention. Did the authors consider holding an additional sessions for faculty or other staff members (i.e., older generations) where they could self-describe their age range?

Two pages are devoted to qualitative analysis of verbal responses according to 4 identifiable themes, i.e. pre-existing stigma, humanization & empathy, increased awareness, and support-oriented perspectives. Importantly, the authors are expansive on the limits of the study, noting the small sample, the risk of selection bias that might lead less positively oriented participants not to complete the post-test survey, as almost half did not do. The authors also note the potential transience of attitudes formed from a brief encounter, and the lack of any evidence of a durable impact. In spite of these limits, the authors rightly note that this simple brief intervention yielded some immediate positive gains that are worth exploring and expanding upon.

We appreciated the inclusion of quotes from participants, but the authors should consider summarizing some of them in the text of the paper and include full quotes in a table. More broadly, the qualitative section is problematic. The chosen quotes are unclear, and the author's inferences as to the speaker's intent are not well explained. This section needs revision, and might better be left to an appendix, or withdrawn entirely. Their contribution to the paper is quite limited.

As written, the discussion is somewhat bulky and might better be organized into sections.

In the discussion, the authors are encouraged to commit a paragraph to the future use of this intervention. How do the authors anticipate this might be received in another setting, e.g. in a hospital lobby, at the entrance of a county courthouse, a Christian college or seminary, or in a senior center? There is considerable global progress in the use of youth sports, e.g. "Coaches Without Borders", as a vehicle for social and political change. How would the authors alter this program for hockey players or soccer players in mind?

The author's conclusions are fairly sound based on what they've described in their analysis of their data.

Reviewer #2: This is my first review of "Reducing Stigma Toward People Who Use Substances Through a Pop-Up Shop Intervention: A Mixed-Methods Pre–Post Study" (PONE-D-26-27576). This manuscript describes a pre-post test design with qualitative interviews assessing changes to public attitudes towards people who use drugs after viewing a harm reduction display with pins, educational games, and other visual/experiential elements intended to reduce stigma. The survey sample consisted of n=39 matched pre-post responses (out of n=70 participants) and the interview sample had n=5 participants. While the intervention appears innovative, its specific elements are not clearly described, limiting the ability for researchers to build on these findings. The significant attitudinal changes are notable, and the qualitative data add some support to the specific attitudinal shifts participants felt, but the qualitative data are very brief and thin in terms of describing experiences and specific mechanisms of change attributable to the specific intervention components. The authors may strengthen the manuscript by dedicating more time to describing the intervention components and, if possible, using their qualitative data to perhaps better point to specific elements of the intervention that interview participants found to shift their beliefs.

Abstract
• The methods section implies that the qualitative data come from open-ended responses to a

survey since surveys are discussed immediately prior and there is no mention of interviews.
• The sample size for qualitative interviews should be mentioned in the results.
Introduction
• While the research on interventions to address public stigma related to drug use is somewhat thin, there have been evaluations of public messaging campaigns about some facets of substance use like naloxone, and I was surprised not to see this mentioned. It would strengthen the manuscript to nod to this work and compare/contrast this intervention with that past work. McGinty's work comes to mind, but others have explored these issues too.
Methods
• In the description of the intervention, it is stated that "public health communication approaches, including historical HIV awareness campaigns and related examples" were used in development. Specific elements of previous approaches that were leveraged, especially any that have been shown as evidence-based communication approaches would help the reader understand what is being referenced here.
• Given that a weakness of many published studies of stigma interventions is that the interventions are essentially a black box, a potential strength to play up in this manuscript is a more detailed and transparent approach to the intervention components and implementation. While some intervention elements are listed (e.g., branded harm reduction kits), more description of the elements of the intervention are needed since the images in appendix A show largely sign-based education, a game, pins, and a small zipped case, but there are mentions elsewhere of a "fashion" and "shop" (were items sold? Given out for free? Which items?) element to the event and storytelling, but it's not clear how these intervention components were implemented. A figure or table describing all the intervention elements or some other representation may be needed to fully grasp how the intervention operated, and having it in the main text is preferable.
• Were study staff "working" at the display, and if so, what did they do/say to participants? Were these conversations part of the intervention approach, or were participants left to experience the content with minimal interaction with the booth attendants/staff?
• The description of the participants does not specify whether these are students, members of the general public, etc. It' s hard to understand who participated in this in terms of demographics, social context. Was this pop up at some sort of event? Or just a regular day on campus?
• Were the attitudes and stigma measures based on any existing scales? If so, these should be mentioned and cited (e.g., Perceived Dangerousness Scale?).
• Same question about the measures assessing acceptability, appropriateness, and feasibility. Were these based on existing measures?
• Did the interviews use a specific methodology? I see mention of "ethnographic style," which maybe means semi-structured or unstructured interviews? Brief intervention evaluation interviews like this would not typically be considered ethnography. If no specific methodology was used, please at least specify whether the approach to qualitative data collection was inductive or deductive in nature.
• Did the interviews use a guide? And what was their length (total minutes recorded and average per interview)? Since there were only n=5, this would give a sense of the depth of the available data. Small sample size is not necessarily an issue in qualitative research, but small sample size combined with very brief interviews does usually limit qualitative inference and this should be more transparent.
Results
• Were any demographics tracked for survey respondents? I assume no, but if so, did people who did not respond to the post survey differ in any way from those who did complete?
• The number of participants in the qualitative sample should be stated at the start of the

qualitative results section.
• Is it possible to determine whether the interview participants were people whose surveys indicated changed attitudes? This may lend greater dependability to the qualitative results.
• The result in which the participant spoke of the "website thing had resources on it" was a clear time when a participant pointed to a specific intervention component that they felt shifted their attitudes, but the authors need to explain what intervention component this is referring to and draw out how they interpret this participant expressing the shift. The text before the quote doesn't seem to really draw out how this person was made aware of specific things (which resources?) through the intervention.
Discussion
• The explanation of how this connects to research on experiential design was interesting, but part of it introduced new information on intervention components (about the interactive digital experience) that would be helpful to have described earlier in the methods/intervention description section.
• Limitations should mention that (seemingly) no demographic, professional, etc. factors were measured, so we do not know the drug use status, age, profession, education, etc. factors that may also influence 1) who stopped at the display and 2) who was willing to respond to the surveys and interviews. We know from past studies that some of these factors are associated with stigmatizing attitudes towards substance use, so not accounting for them may also bias the quantitative results. And since the intervention seems to target general public gen Z members, we don't actually know if the participants were from that target audience.
• Limitations should also explore the depth of the qualitative data and whether more interviews and/or longer interviews would have strengthened the results. I wonder why only 5 participants agreed to interviews? Were more approached and declined?
• Since stigma is more than an attitudinal phenomenon, and has an action/discrimination component, the discussion could be strengthened by exploring how future research building on this work may address this component of stigma. Currently the discussion largely frames stigma as a matter of attitudes/perspectives/beliefs.

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6. PLOS authors have the option to publish the peer review history of their article (what does this mean?). If published, this will include your full peer review and any attached files.

If you choose "no", your identity will remain anonymous but your review may still be made public.

Do you want your identity to be public for this peer review? For information about this choice, including consent withdrawal, please see our Privacy Policy.

Reviewer #1: **Yes:** Renslow Sherer MD

Reviewer #2: No

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